

Daily Content Intel

June 13, 2026

Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, so a network meta-analysis just pooled 139 randomized trials, almost 10,000 people with knee osteoarthritis, and ranked 12 different treatments head to head. Here's what won. Knee bracing came out on top, then water-based exercise, then regular strength and loading work.

And here's the part I care about. Ultrasound and those little lateral wedge insoles, the passive stuff a lot of clinics still bill for, finished near the bottom.

So if your knee hurts and the plan is to lie on a table while a machine buzzes on you for ten minutes, that's probably the least useful thing in the room. The

stuff that actually moved pain, and function, and stiffness was either mechanical, like offloading the joint with a brace, or active, like loading it in the pool or in the gym.

I read this as permission to keep doing what works. I load the joint, I build the quad and the hip, and I use a brace as a bridge while we get strong, not as the whole plan. The joint adapts to load. A sound wave gives it nothing to adapt to.

If your knee's been barking and nobody's had you actually train it, that's the gap. I can help with that.

Be Good. Help Someone. Learn Lots.

Hook: I don't own an ultrasound machine, and a study of almost 10,000 knees just told me why.

Spoken script (word for word):

Okay, so a network meta-analysis just pooled 139 randomized trials, almost 10,000

people with knee osteoarthritis, and ranked 12 different treatments head to head. Here's what won. Knee bracing came out on top, then water-based exercise, then regular strength and loading work.

And here's the part I care about. Ultrasound and those little lateral wedge insoles, the passive stuff a lot of clinics still bill for, finished near the bottom.

So if your knee hurts and the plan is to lie on a table while a machine buzzes on you for ten minutes, that's probably the least useful thing in the room. The stuff that actually moved pain, and function, and stiffness was either mechanical, like offloading the joint with a brace, or active, like loading it in the pool or in the gym.

I read this as permission to keep doing what works. I load the joint, I build the quad and the hip, and I use a brace as a bridge while we get strong, not as the whole plan. The joint adapts to load. A sound wave gives it nothing to adapt to.

If your knee's been barking and nobody's had you actually train it, that's the gap. I can

help with that.

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On-screen text seeds:

- 0:00 I don't own an ultrasound machine
- 0:05 139 trials. ~10,000 knees. 12 treatments.
- 0:15 Winner: bracing > water exercise > strength
- 0:30 Ultrasound + wedge insoles: near the bottom
- 0:45 Passive buzzing won't rebuild a knee
- 1:00 Load the joint. Build the quad + hip.
- 1:10 A brace is a bridge, not the plan

Caption (copy-paste ready):

A network meta-analysis just ranked 12 treatments for knee osteoarthritis across 139 randomized trials and almost 10,000 people.

The winners were bracing, water-based exercise, and strength work. Ultrasound and lateral wedge insoles finished near the bottom.

If your knee hurts and the whole plan is a machine buzzing on a table, you're getting the weakest tool in the room. Knees respond to load and to offloading. Passive modalities barely register.

I load the joint, build the quad and the hip, and use a brace as a bridge while we get strong.

Knee been barking? Let's actually train it.
Book at threshold.clientsecure.me

Dr. Lars Stevenson, PT, DPT

It's time to cross your threshold.

#kneepain #kneeosteoarthritis
#physicaltherapy #strengthtraining
#activeadults #athletes #rehab #prehab
#thresholdhp #restonva

Personalize this

- Pull a real knee OA patient (an active adult or athlete) who came to you after months of ultrasound or modalities elsewhere with no change, then responded once you loaded them. What was the exact loading progression you used, and how fast did pain and function

shift?

- How do you actually use bracing in your practice as a bridge: which brace, for how long, and what's your criteria for weaning it off as their strength comes up?
- Where does "stop paying for passive modalities, start loading" map in your CROSS framework, and how do you say that to a patient without trashing whoever treated them last?

Screenshots:

- title | <https://pmc.ncbi.nlm.nih.gov/articles/PMC12176148/> | "Clinical efficacy of different therapeutic options for knee osteoarthritis: A network meta-analysis based on randomized clinical trials"
- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC12176148/> | "Overall, based on the results obtained from the SUCRA for all outcomes, knee brace had the highest probability of being the best technique, followed by hydrotherapy and exercise."
- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC12176148/> | "The findings

suggest that knee brace may be the most recommended therapeutic option for the knee osteoarthritis, followed by hydrotherapy and exercise."

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC12176148/>
- <https://pubmed.ncbi.nlm.nih.gov/40531843/>
- <https://www.sciencedaily.com/releases/2026/03/260330084511.htm>

Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, hot take. Most of you are optimizing the wrong thing first. You've got the sleep tracking, the fasting window, the gut protocol, the supplement stack that needs its own spreadsheet, and you still can't hit your protein. I see it all the time.

Here's a ranking idea I got from Mike T. Nelson. You score every habit two ways. How big the physiological effect is, times how likely you are to actually do it. Rank everything by that, and protein sits at number one. Sleep, for the record, lands around number eight. Sleep still matters a ton. It's just slow to move and hard to change, so it's a rough place to start.

Protein is the opposite. It's a huge effect and it's easy to do. And the research backs the dose. To max out muscle

protein synthesis you want about 0.4 grams per kilo per meal, across at least four meals, which lands you around 1.6 grams per kilo for the day.

So here's my call. Before you buy another gadget, get 40 grams of protein at four meals and hold it for two weeks. That one move beats the shiny protocol you'll quit by next Thursday.

Be Good. Help Someone. Learn Lots.

Hook: You bought the sleep mask, the glucose monitor, and seven supplements, and you still aren't eating enough protein.

Spoken script (word for word):

Okay, hot take. Most of you are optimizing the wrong thing first. You've got the sleep tracking, the fasting window, the gut protocol, the supplement stack that needs its own spreadsheet, and you still can't hit your protein. I see it all the time.

Here's a ranking idea I got from Mike T. Nelson. You score every habit two ways.

How big the physiological effect is, times how likely you are to actually do it. Rank everything by that, and protein sits at number one. Sleep, for the record, lands around number eight. Sleep still matters a ton. It's just slow to move and hard to change, so it's a rough place to start.

Protein is the opposite. It's a huge effect and it's easy to do. And the research backs the dose. To max out muscle protein synthesis you want about 0.4 grams per kilo per meal, across at least four meals, which lands you around 1.6 grams per kilo for the day.

So here's my call. Before you buy another gadget, get 40 grams of protein at four meals and hold it for two weeks. That one move beats the shiny protocol you'll quit by next Thursday.

Be Good. Help Someone. Learn Lots.

On-screen text seeds:

- 0:00 You optimized everything except protein
- 0:08 Rank habits: size of effect x will you actually do it

- 0:20 Protein = #1. Sleep ~ #8.
- 0:35 Sleep matters. It's just slow to move.
- 0:45 0.4 g/kg per meal, 4 meals
- 0:55 lands ~1.6 g/kg per day
- 1:05 40g x 4 meals. Two weeks. Go.

Caption (copy-paste ready):

Hot take: most people optimize the wrong thing first.

The sleep tracker, the fasting window, the gut protocol, the 17-capsule stack, and they still can't hit their protein.

Rank your habits two ways: how big the effect is, and how likely you are to actually do it. Protein wins. It's a big effect and an easy one. Sleep matters a ton, it's just slow to move and hard to change, so it's a rough first step.

The dose: about 0.4 g/kg per meal, four meals a day, landing near 1.6 g/kg for the day. For a lot of people that's roughly 40 grams, four times a day.

Before the next gadget, hold that for two weeks.

Book at threshold.clientsecure.me

Dr. Lars Stevenson, PT, DPT

It's time to cross your threshold.

#protein #musclebuilding #nutritioncoaching
#strengthtraining #activeadults #athletes
#flexdiet #thresholdhp #restonva
#performance

Personalize this

- Give a real example from your athletes or active-adult clients: someone chasing recovery tech or supplements while under-eating protein. What changed in two weeks once they hit 40g x 4?
- How do you coach the 4x40 in practice? What does breakfast look like for a client who says they can't eat in the morning, and which protein sources do you lean on?
- Where does "start with the highest-leverage habit" fit your CROSS framework and your positioning against the gadget-and-supplement crowd? How do you sequence protein ahead of sleep for a patient who's in pain?

Screenshots:

- title | <https://pmc.ncbi.nlm.nih.gov/articles/PMC5828430/> | "How much protein can the body use in a single meal for muscle-building? Implications for daily protein distribution"
- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC5828430/> | "Based on the current evidence, we conclude that to maximize anabolism one should consume protein at a target intake of 0.4 g/kg/meal"
- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC5828430/> | "across a minimum of four meals in order to reach a minimum of 1.6 g/kg/day"

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC5828430/>
- <https://miketnelson.com/protein1>

Reference

Runner-up topics

- Nordic hamstring exercise injury-prevention RCTs (Feb 2026 Gaelic footballers trial; ~51% hamstring injury reduction pooled) — strong S&C teaching Reel.
- Balance / one-leg-stand as a longevity and CV marker — needs an athlete/active-adult framing to dodge the older-adult demographic trap.
- The contradiction between the Feb 2026 "exercise does little for OA pain" review and the March 2026 network meta-analysis — a nuanced hot take on reading the evidence.

Sources scanned today

- Newsletter digest (2026-06-13): Mike T. Nelson "Coaching Leverage" (protein is intervention #1, sleep is #8, order beats information) — used for Draft B framing. Dr. Mercola (microplastics aging, balance and heart/brain, morning-tiredness) — low-quality/promo, skipped as primary.

- PubMed / PMC: knee osteoarthritis network meta-analysis (139 RCTs, 9,644 patients, 12 therapies) — PMC12176148 / PubMed 40531843. Used for Draft A.
- ScienceDaily (Mar 30 2026): "Simple therapies beat drugs for knee arthritis pain relief" — fresh coverage of the above meta-analysis.
- PMC: Schoenfeld & Aragon per-meal protein paper (0.4 g/kg/meal, 4 meals, 1.6 g/kg/day) — PMC5828430. Used for Draft B research anchor.
- miketnelson.com/protein1 (Flex Diet Cert protein module) — credited in Draft B; page is video-driven, thin on verbatim text, so screenshots anchor on the PMC paper.
- Searched recent Nordic hamstring / plyometric injury-prevention RCTs and balance-mortality studies — mostly older or demographic-targeted (older adults), set aside per audience rules.
Reddit/podcast feeds not individually fetched this run (PubMed + ScienceDaily produced stronger, citable findings).